

Fire Safety and Response (Code Red)

Policy Number	POL-EM-050	Owner / Department	Safety / Emergency Management
Effective Date	2023-04-01	Version	3.2
Last Revised	2026-02-12	Next Review	2028-02-12
Approved By	Environment of Care Committee	Applies To	All staff, licensed providers, students, volunteers, and contractors, in all buildings and on all shifts

1. Purpose

To protect life and property through early fire detection, prompt notification, confinement, and — only when necessary — extinguishment or evacuation, consistent with the NFPA 101 Life Safety Code and The Joint Commission Environment of Care and Emergency Management standards. Riverside Regional Medical Center is a fully sprinklered, compartmentalized facility and follows a **defend-in-place** strategy: in a fire, patients are protected behind fire/smoke barriers rather than immediately removed from the building.

2. Scope

Applies to any fire, smoke, or odor of burning discovered in any area of the facility, and to the actions of every person present regardless of role. Staff who discover a fire act immediately; they do not wait for the fire alarm or for Security.

3. Definitions

- **Code Red** — the overhead alert announcing a fire or suspected fire, followed by the specific location.
- **RACE** — the immediate-action sequence: Rescue, Alarm, Confine, Extinguish or Evacuate.
- **PASS** — the fire-extinguisher technique: Pull, Aim, Squeeze, Sweep.
- **Defend in place** — protecting occupants behind rated fire/smoke barriers (closing doors, moving patients to an adjacent smoke compartment) instead of leaving the building.
- **Smoke compartment** — an area bounded by smoke-resistant barriers and self-closing doors into which occupants can be moved horizontally.

4. Procedure — RACE

IMPORTANT — RACE: Rescue, Alarm, Confine, Extinguish/Evacuate. Rescue anyone in immediate danger, pull the nearest fire alarm and call the emergency number to announce Code Red with the exact location, close doors to confine smoke and fire, then extinguish only a small incipient fire or evacuate. Never use elevators during a Code Red.

1. **R – Rescue.** Remove any person in immediate danger from the fire or smoke to a safe adjacent area.

2. **A – Alarm.** Activate the nearest manual pull station and call the emergency number to report a Code Red and the exact location; do not assume someone else has called.
3. **C – Confine.** Close all doors and windows to contain smoke and fire; clear corridors of equipment.
4. **E – Extinguish or Evacuate.** If the fire is small, incipient, and you are trained, use the nearest extinguisher with the PASS technique; otherwise close the door and evacuate horizontally to the next smoke compartment.

Evacuation order (when required): move occupants **horizontally** through smoke-barrier doors to an adjacent compartment first; use **vertical** (stairwell) evacuation to a lower floor only if the compartment is untenable; **full building evacuation** is a last resort directed by the Incident Commander or the fire department.

5. Evacuation Priority

When an area must be cleared, evacuate in order of mobility and proximity to the fire. The least mobile patients are moved first only when they are in the immediate path of fire or smoke; otherwise ambulatory patients are moved first to clear the egress path.

Priority	Group	Typical method
1	Occupants in the room of fire origin and the immediately adjacent rooms.	Direct carry/assist to the next smoke compartment.
2	Ambulatory patients in the affected compartment.	Walk, staff-escorted, through barrier doors.
3	Wheelchair / assisted-mobility patients.	Wheelchair or transport chair.
4	Non-ambulatory / critical patients (ICU, OR, ventilated).	Bed, stretcher, or rescue-sheet drag; staff accompany life-support equipment.

6. Severity & Reporting

Fire events are classified under POL-RM-013. A fire causing injury, requiring patient evacuation, or resulting in a fire department response with suppression is **SEV-1**: notify the Administrator-on-Call, Safety Officer, and Facilities immediately and within 1 hour, and activate the Hospital Incident Command System (HICS) if the event extends beyond a single compartment. A contained incipient fire extinguished without injury or evacuation is **SEV-2** (notify Safety and Facilities within 4 hours). A fire-alarm activation with no fire, or a code-correctable fire-safety deficiency, is **SEV-3 / Near Miss**, reported within 24 hours. File an event report for every Code Red, including false alarms and cooking/steam nuisance activations.

7. Documentation Requirements

- Exact location, time of discovery, and how the fire or alarm was detected.
- Whether the alarm was activated and the time the fire department was notified / arrived.
- Actions taken (RACE steps, extinguisher used, doors closed, evacuation performed and to where).
- Any injuries, patient moves, and equipment or structural damage.
- Notifications made (who, when, by what method) and HICS activation status.

8. References

- NFPA 101, Life Safety Code — health care occupancies (defend-in-place, smoke compartments, egress).
- NFPA 99, Health Care Facilities Code; NFPA 10 — portable fire extinguishers.

- The Joint Commission, Environment of Care (EC.02.03.01) and Emergency Management standards; Life Safety (LS) chapter.
- CMS Emergency Preparedness Rule, 42 CFR 482.15.
- Related: POL-RM-013 (severity & reporting), POL-EM-053 (HICS / Code Triage), POL-EM-054 (utility failure).